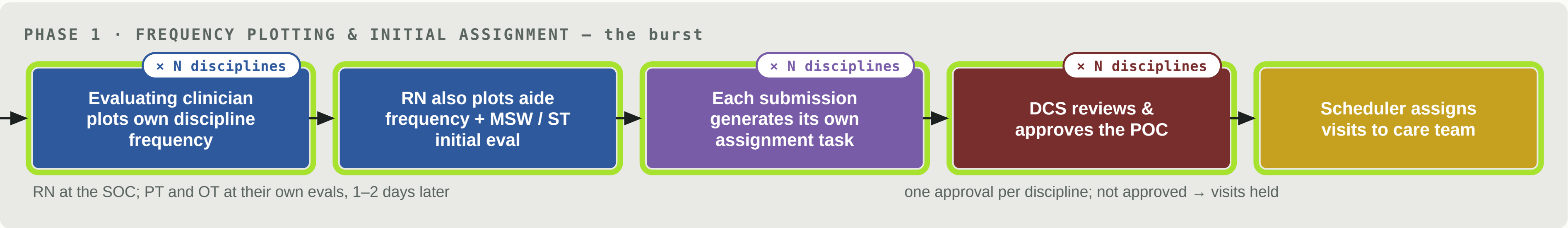
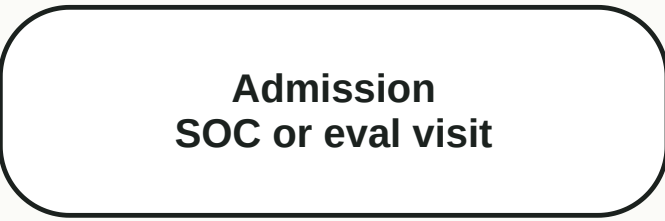


Routine Visit Scheduling

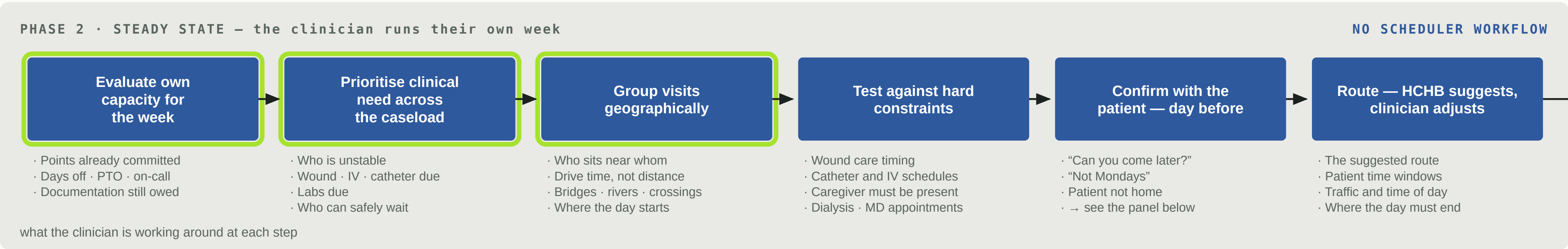
A burst of scheduler work at admission, then the clinician runs their own week

TRIGGER



485 SUBMITTED – changes come by physician order

a physician order may add a discipline evaluation, change the frequency, or reduce the visits every one routes through DCS approval and auth before reaching the scheduler



THE DAY-BEFORE NEGOTIATION – what the clinician has to hold

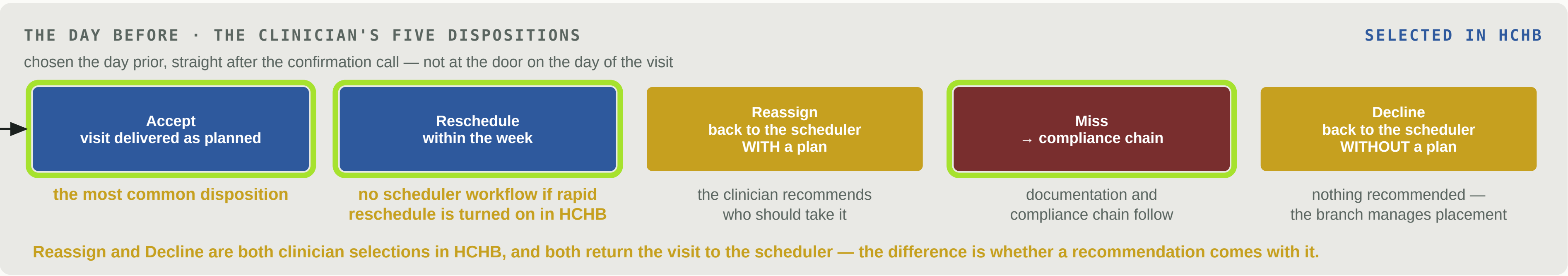
HARD – accept it and build the day around it

- Dialysis days and times
- MD and specialist appointments
- Caregiver's working hours
- Patient genuinely not home

SOFT – negotiable, and worth holding the line on

- “Can you come after lunch?”
- “Not first thing” · “Not Mondays”
- A preferred time with no reason behind it

The first visit at 8 or 9am is the single largest lever on an individual clinician's capacity. Newer clinicians let the patient set the time, become over-accommodating, and push the cost onto the rest of the team.



BOUNDARIES



WHERE IT
BREAKS

